GULF COAST VASCULAR SURGERY ASSOCIATES

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PATIENT MEDICAL CHART — CONFIDENTIAL — FOR AUTHORIZED USE ONLY

1. PATIENT DEMOGRAPHICS

Patient Name	Maria Elena Delgado
Date of Birth	June 12, 1971
MRN	GCV-010442
Age	54 years
Sex	Female
Address	1410 Palmetto Ave, Tampa, FL 33605
Primary Care Physician	Dr. Samuel Ortiz, MD

2. INSURANCE & COVERAGE

Primary Insurance

Payer	Florida Blue (Blue Cross and Blue Shield of Florida)
Plan Type	PPO — BlueOptions
Member ID	FLB442918733
Group Number	FL-55120-OPT
Coverage Effective	January 1, 2025
Prior Auth Required	Yes — Vein procedures
Auth Contact	1-800-555-0199

Secondary Insurance

None on file.

3. ACTIVE PROBLEM LIST

ICD-10	Diagnosis	Onset	Status
I83.813	Varicose veins of bilateral lower extremities with pain	2024	Active
I87.2	Venous insufficiency (chronic) (peripheral)	2024	Active
E66.9	Obesity, unspecified (BMI 31)	2019	Active

4. CURRENT MEDICATIONS

Medication	Dose	Frequency	Indication
Ibuprofen	400 mg	PRN	Leg pain
Losartan	50 mg	Daily	Hypertension

5. ALLERGIES & ADVERSE REACTIONS

No known drug allergies.

6. VISIT NOTES — VASCULAR SURGERY

Visit Date: March 18, 2025 | Attending: Dr. Elena Vasquez, MD, FACS | Type: New Patient Consult

Chief Complaint:

Left leg aching, heaviness and swelling, worse by end of shift. Works 10-hour shifts as a hospital unit clerk, standing most of the day.

History of Present Illness:

54-year-old female with 18 months of progressive left lower extremity aching, heaviness, nocturnal cramping and ankle swelling. Symptoms worsen with prolonged standing and improve with elevation. Reports itching over medial calf with new brownish discoloration. Denies history of DVT, ulceration, bleeding from veins, or superficial thrombophlebitis. Symptoms now limit her ability to complete shifts; she has used 6 sick days in 3 months for leg pain and has stopped evening walks.

Physical Examination:

Left leg: prominent tortuous varicosities along the medial thigh and calf in the GSV distribution, largest cluster approx. 6 mm diameter on exam. Hyperpigmentation and mild eczematous change medial lower calf. 1+ pitting edema at ankle. No ulceration. Pulses 2+. Right leg: scattered reticular veins only.

Assessment & Plan:

Symptomatic chronic venous insufficiency, left leg, CEAP clinical class C4a (pigmentation/eczema). Order lower extremity venous reflux duplex. Begin conservative management: graduated compression stockings 20–30 mmHg daily, leg elevation, NSAIDs PRN, weight management counseling. Return 3 months to assess response.

Electronically signed: Dr. Elena Vasquez, MD, FACS — March 18, 2025

Visit Date: July 22, 2025 | Attending: Dr. Elena Vasquez, MD, FACS | Type: Follow-Up

Interval History:

Patient returns after 4 months of conservative therapy. Reports consistent daily use of 20–30 mmHg compression stockings since the March visit (confirms wearing them to work every shift), nightly elevation, and ibuprofen PRN. Symptoms unchanged to slightly worse: persistent aching and heaviness by mid-shift, ankle swelling every evening, nocturnal cramps 3–4 nights per week. Itching persists. She reports she cannot stand for a full shift without sitting breaks and has reduced her hours. Duplex reviewed below.

Physical Examination:

Unchanged varicosities medial left thigh and calf, 5–6 mm. Hyperpigmentation medial calf stable. 1+ edema. No ulcer.

Assessment & Plan:

Symptomatic left GSV reflux with CEAP C4a disease, failed 4 months of compression therapy and conservative measures. Symptoms significantly interfere with work and daily activities. Recommend endovenous radiofrequency ablation of the left great saphenous vein. Risks, benefits, alternatives discussed; patient wishes to proceed. Submit prior authorization to Florida Blue.

Electronically signed: Dr. Elena Vasquez, MD, FACS — July 22, 2025

7. DIAGNOSTIC IMAGING & TESTING REPORTS

Lower Extremity Venous Reflux Duplex Ultrasound — Left — March 25, 2025

Ordering Physician	Dr. Elena Vasquez, MD, FACS
Indication	Symptomatic varicose veins, left leg; evaluate for reflux
Deep System	Common femoral, femoral, popliteal veins compressible, no DVT. No deep venous reflux.
Great Saphenous Vein	Incompetent. Reflux at saphenofemoral junction 2.4 seconds with Valsalva; reflux along thigh and calf segments 1.1–1.8 seconds on distal compression release. GSV diameter 7.8 mm at SFJ, 6.1 mm mid-thigh, 4.9 mm at knee.
Small Saphenous Vein	Competent. No reflux. Diameter 2.8 mm.
Perforators	One incompetent medial calf perforator, 3.4 mm, reflux 0.8 s.
Tributaries	Multiple dilated tributary varicosities medial calf, 4–6 mm.

Interpretation: Left great saphenous vein incompetence with significant axial reflux (>0.5 s) from the SFJ through the calf. No deep venous thrombosis or deep reflux. Findings consistent with symptomatic superficial venous insufficiency.

Electronically signed: Dr. Thomas Reed, RVT / Dr. Elena Vasquez, MD — March 25, 2025

8. PLANNED PROCEDURE

Planned Procedure	Endovenous radiofrequency ablation, left great saphenous vein
CPT	36475 — Endovenous ablation therapy of incompetent vein, extremity, radiofrequency; first vein treated
Laterality	Left
Diagnosis	I83.813, I87.2
Surgeon	Dr. Elena Vasquez, MD, FACS
Facility	Gulf Coast Vascular Surgery Associates — office-based procedure suite
Payer	Florida Blue PPO
PA Status	Pending — Submitted July 24, 2025

Clinical Indication Summary:

• Symptomatic left GSV reflux (SFJ reflux 2.4 s) on duplex 03/25/2025; no DVT.

• CEAP clinical class C4a: varicosities with pigmentation and eczema.

• Varicosities 5–6 mm on exam; GSV 7.8 mm at SFJ.

• Conservative management with 20–30 mmHg compression, elevation and NSAIDs from 03/18/2025 through 07/22/2025 (4 months) without improvement.

• Symptoms interfere with occupation (standing shifts) and daily activities.

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